

HOUSE No. 2499

The Commonwealth of Massachusetts

PRESENTED BY:

Joan Meschino

To the Honorable Senate and House of Representatives of the Commonwealth of Massachusetts in General Court assembled:

The undersigned legislators and/or citizens respectfully petition for the adoption of the accompanying bill:

An Act pertaining to women's health at midlife and public, medical and workplace awareness of the transitional stage of menopause and related chronic conditions.

PETITION OF:

NAME:	DISTRICT/ADDRESS:	DATE ADDED:
<i>Joan Meschino</i>	<i>3rd Plymouth</i>	<i>1/8/2025</i>
<i>Patrick Joseph Kearney</i>	<i>4th Plymouth</i>	<i>1/8/2025</i>
<i>Michael D. Brady</i>	<i>Second Plymouth and Norfolk</i>	<i>3/27/2025</i>
<i>Mike Connolly</i>	<i>26th Middlesex</i>	<i>4/1/2025</i>
<i>Natalie M. Higgins</i>	<i>4th Worcester</i>	<i>8/6/2025</i>
<i>Vanna Howard</i>	<i>17th Middlesex</i>	<i>4/3/2025</i>
<i>Hadley Luddy</i>	<i>4th Barnstable</i>	<i>7/7/2025</i>
<i>Margaret R. Scarsdale</i>	<i>1st Middlesex</i>	<i>1/9/2026</i>
<i>Bruce E. Tarr</i>	<i>First Essex and Middlesex</i>	<i>4/2/2025</i>

HOUSE No. 2499

By Representative Meschino of Hull, a petition (accompanied by bill, House, No. 2499) of Joan Meschino and Patrick Joseph Kearney relative to public, medical and workplace awareness of the transitional stage of menopause and related chronic conditions. Public Health.

The Commonwealth of Massachusetts

In the One Hundred and Ninety-Fourth General Court
(2025-2026)

An Act pertaining to women’s health at midlife and public, medical and workplace awareness of the transitional stage of menopause and related chronic conditions.

Be it enacted by the Senate and House of Representatives in General Court assembled, and by the authority of the same, as follows:

1 SECTION 1. The General Court hereby declares and finds that:

2 (i) by 2030, the world population of menopausal and postmenopausal women is
3 projected to increase to 1.2 billion, with 27 million new entrants each year;

4 (ii) each year, women in the United States enter the menopausal transition with little
5 clinical guidance on what to expect during and after this transition;

6 (iii) according to the United States Department of Health and Human Services, at least 3
7 out of 4 women experience hot flashes, the most common menopause symptom, and other
8 symptoms including memory loss, urinary problems, sleep disturbances, depression and anxiety;

9 (iv) menopausal symptoms can be severe and affect daily activities and quality of life
10 for an extended period, with hot flashes lasting an average of 7 to 9 years, and a third of women
11 experiencing vasomotor symptoms for a decade or longer;

(v) studies show that Black and Hispanic women may experience menopause earlier, with more intense menopausal symptoms, and for a longer period of time;

(vi) as many as 40 per cent of menopausal women say their symptoms interfered with their work performance or productivity weekly, and nearly 1 in 5 say they have left or considered leaving the workforce because of their symptoms;

(vii) many women are unsure what accommodations their employers offer for menopause and are unsure about workplace culture regarding menopause;

(viii) menopause symptoms cost American women an estimated \$1.8 billion in lost working time per year;

(ix) due to medical innovation, a variety of effective treatments for symptoms are available to help address symptoms during perimenopause and menopause, including, but not limited to, hormone therapy and nonhormone medication;

(x) according to the United States Department of Health and Human Services, menopause may increase the risk of osteoporosis, heart disease and stroke;

(xi) there is a need for additional clinical research and treatment options to manage menopause symptoms;

(xii) many physicians, including obstetricians and gynecologists, have limited time to assess menopause symptoms during visits with patients; and

(xiii) many physicians have limited training on menopause, and only approximately 30 per cent of obstetrician and gynecology residency program directors report that menopause curriculum is part of resident training.

SECTION 2. For the purposes of this act, the following terms shall, unless the context clearly requires otherwise, have the following meanings:

“Board”, the board of registration in medicine, established in section 10 of chapter 13 of the General Laws.

“CME”, a continuing medical education requirement for physicians, as required by the board pursuant section 2 of chapter 112 of the General Laws and any regulations promulgated thereunder.

“Department”, the department of public health.

“Executive office”, the executive office of labor and workforce development.

SECTION 3. (a)(1) Not later than 270 days following the passage of this act, the commissioner of the department, in consultation with clinical practitioners and nonprofit organizations that promote the health of women during menopause, shall develop and carry out a program to improve patient and clinician awareness of the menopause transition.

(2) The program shall provide for the development, publication and dissemination of materials appropriate for the education of patients and clinicians that describe:

(i) symptoms and trajectories of changes across the menopausal transition and the post-menopause transition;

(ii) related chronic conditions; and

(iii) the entire range of treatment options that may be prescribed by a health care provider for those symptoms, changes and conditions, as well as available screening tools.

(3) A menopause education program may include:

(i) the use of social media, television, radio, print, the internet and other media; and

(ii) public service announcements and in-person or interactive virtual public communications.

(b) Not later than 18 months following the passage of this act, the department shall submit to the general court a qualitative assessment of the education program described in this section and a description of the activities conducted thereunder.

SECTION 4. (a)(1) Not later than 270 days following the passage of this act, the board shall assess whether the CMEs currently available to physicians practicing in the commonwealth are adequate to train providers on women's midlife health.

(2) If the board finds that CME programs available to physicians practicing in the commonwealth do not provide adequate training and education regarding women's midlife health, the board shall take appropriate steps to ensure the adequacy of programs available to physicians on these subjects.

(b) The board may, subject to appropriation, award grants to accredited CME providers located in the commonwealth to be used for any of the following purposes:

(i) training, including for individuals completing a residency, fellowship or other clinical training for licensure, for physicians in order to improve communication and provider preparedness in the management of menopausal symptoms and related chronic conditions;

(ii) establishing, maintaining or improving academic units or programs that provide training for students of faculty, including clinical experience and research, to improve the ability to recognize, diagnose and treat menopause symptoms and related chronic conditions; or

(iii) developing evidence-based practices or recommendations for the design of programs for education on menopause symptoms and related chronic conditions.

SECTION 5. (a) Not later than 270 days following the passage of this act, the executive office shall, in consultation with the department, conduct a study to assess the impact of menopause on the workforce and the breadth of menopause-related workplace policies offered by employers in the commonwealth, including, but not limited to, health insurance coverage of therapeutics for menopause symptoms, access to menopause health care professionals, menopause awareness policies, healthcare spending accounts that can be used for menopause related services and cooling rooms.

(b) Not later than 60 days following the conclusion of the study required in subsection (a), the executive office shall publish the results of its study on its website.

(c) Based on the results of the study described in this section, and within 60 days of the study's publication, the executive office shall develop and publish on its website best workplace practices that include the following elements:

(i) employee benefits for peri- and post-menopausal employees;

(ii) appropriate company culture for peri- and post- menopausal employees;

(iii) programs that increase awareness in the workplace of the menopause transition and related symptoms and chronic conditions.